VCRSoft Consultation Transcript

Company: Lone Star Family Dental

Business type: Dental Clinic

Engagement: 1

VCRSoft

Hi David Kim — I'm an AI consultant that can help you leverage AI tools and solutions to grow Lone Star Family Dental. Let's start: how many locations do you operate, how many dentists, hygienists, and operatories do you have, and what kinds of dentistry do you provide?

YOU — Turn 1

We have 2 locations: our main office in Austin and a second office in Cedar Park, Texas. Austin has 6 operatories — 4 restorative and 2 hygiene — and Cedar Park has 4 operatories — 2 restorative and 2 hygiene — so 10 operatories total. We have 3 dentists: myself as owner, plus 2 associates — Dr. Alvarez, who works 3 days a week at Austin, and Dr. Patel, who works 2 days a week at Cedar Park. We have 3 hygienists — 2 in Austin and 1 in Cedar Park. We provide general dentistry, cosmetic dentistry, Invisalign, hygiene, and same-day emergency slots. We refer out implants, endodontics, and oral surgery. We have Spanish-speaking staff at both locations.

VCRSoft

Running two locations with ten operatories and Spanish-speaking staff gives you a solid footprint across Austin and Cedar Park. What are your roughly estimated annual revenue, total active patients, and average monthly visit volume?

YOU — Turn 2

Our annual revenue is $1,850,000 in collections, about $154,200 per month — roughly $1,150,000 from Austin and $700,000 from Cedar Park. We have about 2,100 active patients, meaning patients seen in the last 18 months. We average about 330 patient visits per month — about 210 at Austin and 120 at Cedar Park.

VCRSoft

Averaging $1,850,000 in collections across 2,100 active patients and 330 monthly visits gives us a clear financial and volume baseline for your two locations. What customer or service segments matter most to your practice's revenue or growth?

YOU — Turn 3

By revenue, our service lines are restorative 42%, hygiene 32%, Invisalign 12%, exams and X-rays 8%, and cosmetic 6%. Invisalign matters most for growth — it has the highest margin per hour, and we do about 6 starts a month, constrained by consult slots. Hygiene is also critical — it is 32% of revenue and has the best margin per chair-hour after labour, but it is constrained by hygienist capacity. On the patient side, our payer mix is 52% PPO, 30% fee-for-service, and 18% other, including our discount plan and a little Medicaid. Patients choose us for gentle care, Spanish-speaking staff, and same-day emergency slots.

VCRSoft

Generating $1,850,000 annually with strong margins in Invisalign and hygiene gives us a clear operational picture. What is your primary growth objective, what measurable target would represent success, and by when?

YOU — Turn 4

Our primary growth objective is to grow collections from $1,850,000 a year to $2,400,000 a year — about $200,000 a month — within 12 months. Success means adding about 18 new patients a month, going from 68 to 86 completed first visits per month, plus reactivating our overdue recare patients.

VCRSoft

Targeting $2,400,000 in collections by adding 18 new patient visits a month gives us a concrete revenue and volume goal for the year. What monthly operating budget and one-time setup budget would you consider for an improvement if the expected return is clear?

YOU — Turn 5

For an improvement with a clear return, I would consider $2,000 to $3,500 a month in operating budget, and up to $8,000 as a one-time setup cost. I would expect payback within 6 months.

VCRSoft

A monthly budget of $2,000 to $3,500 with a 6-month payback gives us clear financial parameters for investing in your growth goals. How many hours per week do you spend on clinical delivery and day-to-day operations versus management, and what would your ideal split look like?

YOU — Turn 6

I spend 32 hours a week on clinical work and 10 hours a week on admin and management. My ideal split would be 30 hours clinical and 5 hours admin. The time I free up would go to Invisalign cases and mentoring my two associates, Dr. Alvarez and Dr. Patel.

VCRSoft

Spending 32 hours weekly on clinical work and wanting to shift that time toward Invisalign cases and mentoring associates gives us a clear view of your personal capacity constraints. Are you facing a major decision in the next year—such as expansion, a lease, equipment, hiring, or a significant contract—and what are its terms and timing?

YOU — Turn 7

Yes, three decisions are pending. First, our Cedar Park lease renewal comes up in 7 months — rent is $4,800 a month now and the landlord is proposing a 6% increase to about $5,088 a month. Second, in 3 months I need to decide whether to hire a second hygienist at Cedar Park or extend Dr. Patel from 2 to 4 days a week there. Third, I am deciding whether to add 2 operatories at Austin next year. A third site is not on the table yet — I would only expand if collections hold a $2.4 million run rate for 2 quarters, chair utilisation tops 88%, the hygiene waitlist tops 75 patients, and our cash reserve reaches $150,000.

VCRSoft

Evaluating the Cedar Park lease renewal alongside your strict expansion gates gives us a clear look at your facility planning. Where do your new patient inquiries currently come from, and what share of those inquiries successfully convert into booked first visits?

YOU — Turn 8

We get about 120 new patient inquiries a month. About 78 of them book a first visit — a 65% conversion rate — and about 68 complete it, which is 87% of those booked. By source, patient referrals are 32%, Google search 28%, Google Ads 18%, insurance directories 12%, and social media 8%.

VCRSoft

Knowing that patient referrals make up 38 of your 120 monthly inquiries while Google Ads bring in about 22 gives us a clear look at your acquisition channels. For those 42 inquiries each month that do not successfully book a first visit, what are the primary reasons you hear for dropping out, and what follow-up happens with them?

YOU — Turn 9

The main reasons inquiries drop out are price or no insurance, 22%, out-of-network insurance, 22%, the hygiene wait, 18%, and no callback reached, 14%, with other reasons making up the remaining 24%. Our follow-up is one callback plus one text, then nothing — we have no nurture sequence for them.

VCRSoft

Losing 18% of unbooked inquiries to hygiene wait times points right to how access and continuity are managed. Where do patients experience delays or drop-offs between their first contact, service start, follow-up, and completion across your two locations?

YOU — Turn 10

The biggest delays are at first contact and in hygiene access. We respond within 2 business hours during opening hours, but about 22% of inquiries arrive outside business hours and there is no automated after-hours response, so they wait until the front desk follows up the next business morning — an average of about 14 hours. Cedar Park has no online scheduling, while Austin does. Intake is digital at Austin, about 8 minutes per patient, but paper at Cedar Park, about 12 minutes, and insurance verification takes 24 hours before the visit. Hygiene is booked 3 weeks out and the dentist 5 days out. No-shows are 9% for hygiene and 6% for the dentist, and about 20% of reminders fail because of a wrong number or no answer. On treatment plans, follow-up is one call only, and we have 230 unscheduled plans worth about $290,000. On recare, 520 patients are overdue and the front desk only manages about 25 calls a week.

VCRSoft

With 520 patients overdue for recare and only 25 calls a week managed by the front desk, continuing care volume is leaking significantly. Which specific referral sources or professional networks generate meaningful volume for you, and how do you track their quality and conversion?

YOU — Turn 11

Patient referrals are our biggest referral source — about 32% of new patients. We track them through the 'how did you hear' field in Dentrix, but it is only completed about 70% of the time. Professionally, we refer out to endodontics, oral surgery, and orthodontics, but we have no formal reciprocal tracking, and our relationship with one orthodontics practice is weakening. Over the last 6 months, Google Ads are up slightly, referrals are flat, and insurance directories are down.

VCRSoft

Patient referrals making up nearly a third of your new business while the Dentrix tracking field is missed a quarter of the time highlights a gap in attribution. What do you spend on marketing or advertising, and can you connect that spend to qualified leads, customers, or revenue?

YOU — Turn 12

We spend $2,600 a month on marketing — $1,200 on Google Ads, $800 on SEO, and $600 on social media. Google Ads brings about 34 inquiries a month at about $35 each, so that spend is traceable. Referrals cost us nothing. SEO cannot be tied to revenue precisely.

VCRSoft

Spending $1,200 on Google Ads to bring in 34 traceable inquiries gives us a clear baseline for paid acquisition cost. Which local competitors or alternative care providers do your prospective patients most frequently compare you with, and what do you find makes them choose Lone Star Family Dental over those options?

YOU — Turn 13

The main competitor patients compare us with is Aspen Dental, about 2 miles from our Austin office, plus 3 independent practices nearby. Patients choose us for gentle care, Spanish-speaking staff, and same-day emergency slots. We lose to the DSO on price, on hygiene wait time, and on Saturday hours at Cedar Park, since Cedar Park is closed on Saturdays.

VCRSoft

Losing out on Saturday hours and hygiene wait times at Cedar Park highlights where patient leakage happens against local competitors. After delivering a service, how consistently do your teams request online reviews, maintain touchpoints with patients, or run structured outreach to bring past patients back?

YOU — Turn 14

We have a 4.6 Google rating from 310 reviews, but we only ask for reviews about 30% of the time, mostly after Invisalign or emergency visits — there is no systematic review request process. We have no re-engagement campaign for lapsed patients beyond recall calls. On recare, 520 patients are overdue and the front desk calls about 25 overdue patients a week from one manual phone list, with about 20% rebooking.

VCRSoft

Calling 25 overdue recare patients a week from a manual list while 520 sit overdue highlights a major capture gap in your hygiene pipeline. What are your total monthly collections, and how do those collections break down across your major service lines?

YOU — Turn 15

Our total monthly collections are about $154,200, which is $1,850,000 a year. By service line, that breaks down as restorative 42%, about $64,764 a month; hygiene 32%, about $49,344 a month; Invisalign 12%, about $18,504 a month; exams and X-rays 8%, about $12,336 a month; and cosmetic 6%, about $9,252 a month.

VCRSoft

With hygiene generating about $49,344 a month at your best margin per chair-hour, understanding how your scheduled hours compare to actual production will show where capacity is constrained. How do your available dentist and hygienist hours compare to your used chair hours across your 10 operatories by day and time, and what specific factors cause unused chair or operatory time?

YOU — Turn 16

Hygiene chairs are 85% booked and dentist chairs are 72% booked. Hygiene is booked 3 weeks out and the dentist 5 days out. The Austin Saturday half-day is full, but Monday and Friday have 20–30% gaps at Cedar Park. No-shows cause unused time — 9% for hygiene and 6% for the dentist — and when a cancellation happens the front desk calls the recall list and fills about 45% of those slots. The biggest constraint is hygienists: we have a hygiene waitlist of 40 patients, an Invisalign consult waitlist of 8, and a part-time hygienist role at Cedar Park has been open for 4 months.

VCRSoft

A part-time hygienist role at Cedar Park being open for four months directly compounds your hygiene waitlist of 40 patients. What specific factors make recruiting hygienists difficult for your Cedar Park location, and how does that open role impact the current workload and scheduling for your existing hygiene staff?

YOU — Turn 17

The open role is a part-time hygienist position at Cedar Park, and it has been open for 4 months. That leaves Cedar Park with just 1 hygienist covering its 2 hygiene operatories, while Austin has 2 hygienists. The impact is direct: hygiene chairs are 85% booked, hygiene is booked 3 weeks out, and we have a hygiene waitlist of 40 patients. The existing hygienists carry the full hygiene load, and hygienist capacity is what constrains our

growth most.

VCRSoft

Operating Cedar Park with one hygienist across two hygiene operatories directly creates the three-week booking wait and forty-patient waitlist. Which specific professional roles, credentialed staff, or room equipment constraints limit your day-to-day service capacity beyond hygiene staffing, and how does your actual patient demand compare with available capacity by service type?

YOU — Turn 18

Beyond hygiene, our capacity limits are staffing depth. We have 1 dental assistant and 1 treatment coordinator across both sites, and 3 front-desk staff — we lost 2 front-desk people in the last year, one per site. Our office manager Rachel Torres holds most of our process knowledge. Our associates are part-time: Dr. Alvarez works 3 days a week at Austin and Dr. Patel 2 days a week at Cedar Park. On demand versus capacity: hygiene demand exceeds capacity — hygiene chairs are 85% booked, booked 3 weeks out, with a 40-patient waitlist — while dentist chairs are 72% booked and booked 5 days out. Invisalign consults have a waitlist of 8. The Austin Saturday half-day is full, but Monday and Friday have 20–30% gaps at Cedar Park.

VCRSoft

Having three front-desk staff and losing two people in the last year creates a heavy operational load on your office manager Rachel Torres. Where do onboarding, order-entry, or verification errors usually occur across your two locations, and when are those errors typically discovered?

YOU — Turn 19

The main errors are in insurance verification. It takes the front desk 2 to 2.5 hours a day, and we get 6 to 8 eligibility denials a month, usually a wrong plan or expired eligibility. They are typically discovered at check-in or when a claim is denied, and each denial costs about $175 in staff rework and delayed payment. Intake data entry also takes about 45 minutes a day across both sites. The two sites differ: TCPA consent is recorded in Dentrix at Austin but is paper-only at Cedar Park and not systematically recorded, and our Dentrix reports have to be exported to CSV manually — the information is not easily accessible across the two sites, so Rachel consolidates the weekly numbers by hand.

VCRSoft

Consolidating weekly numbers by hand from manual CSV exports across your two locations creates significant administrative friction for Rachel Torres. How do you handle patient invoicing and payment collections across your two sites, what share of patient balances are delayed or disputed past 60 days, and how much staff time does the process require each week?

YOU — Turn 20

We collect the patient portion at checkout about 82% of the time, and the remainder is billed monthly. Collections run 95% within 30 days, 3% in 31 to 60 days, and 2% over 90 days. Our AR over 90 days is about $22,000. Disputes are rare, under 3 a month, and are usually about the insurance estimate versus the actual amount. On staff time, insurance verification alone takes the front desk 2 to 2.5 hours a day, and claim follow-up is one of the repetitive tasks eating the most time across both sites.

VCRSoft

Collecting 82% of patient portions at checkout while keeping 95% of accounts current within 30 days shows a tight billing process, even with $22,000 sitting in accounts receivable over 90 days. What are the most frequent reasons claims or required approvals are delayed, rejected, or sent back for rework by insurance payers?

YOU — Turn 21

The most frequent reasons are eligibility problems: 6 to 8 eligibility denials a month, usually a wrong plan or expired eligibility, discovered at check-in or when a claim is denied. Each one costs about $175 in staff rework and delayed payment. Disputes are rare — under 3 a month — and are usually the insurance estimate versus the actual amount. Overall, adjustments and write-offs run about 20% of production.

VCRSoft

Adjustments and write-offs running at 20% of production shape your net collections significantly. What tools or communication methods do you currently use for tasks like patient intake, appointment reminders, and follow-up, and which of those tasks consume the most staff time?

YOU — Turn 22

We use Dentrix for practice management, Dexis for imaging, and QuickBooks Online for finance. Intake is digital at Austin, about 8 minutes per patient, and paper at Cedar Park, about 12 minutes. Online scheduling is available at Austin only. Reminders are a manual phone call the day before, and SMS only if the patient opted in — about 20% of reminders fail because of a wrong number or no answer. We have no automated recall at either site and no AI tools at all. The tasks consuming the most staff time are insurance verification at 2 to 2.5 hours a day, reminder calls at about 1.5 hours a day, recall calls, claim follow-up, and intake data entry at about 45 minutes a day.

VCRSoft

Insurance verification and reminder calls take up several hours of your team's day across both sites. Which provider roles or specialties are currently limiting your overall patient volume or appointment availability?

YOU — Turn 23

Hygienists limit us most — we have only 3 hygienists for 10 operatories, the part-time hygienist role at Cedar Park has been open 4 months, hygiene is booked 3 weeks out, and we have a 40-patient hygiene waitlist. Next, associate availability: Dr. Alvarez works only 3 days a week at Austin and Dr. Patel only 2 days a week at Cedar Park. Invisalign is also limited by consult slots — we do about 6 starts a month with a consult waitlist of 8.

VCRSoft

Having hygienists booked three weeks out with a forty-patient waitlist makes hygiene your primary capacity bottleneck. Looking at your operating expenses, how do staff wages and lab fees scale relative to your monthly collections across both locations?

YOU — Turn 24

Our overhead runs about 68% of collections, which is about $104,856 a month on $154,200 in monthly collections. Staff wages and lab fees sit inside that overhead, so they scale with collections at that 68% rate. Our profit margin is about 18%, about $333,000 a year, or about $27,750 a month.

VCRSoft

With an overhead of 68% and an 18% profit margin, your financial baseline is clear. What is the average revenue per active patient or per visit in the practice?

YOU — Turn 25

Our average revenue is about $881 per active patient per year — $1,850,000 across 2,100 active patients — and about $467 per visit, based on $154,200 in monthly collections across about 330 visits a month.

VCRSoft

Your average revenue of $881 per active patient and $467 per visit give us a clear unit-economics baseline. What is the average duration of a standard hygiene recall appointment versus a restorative filling at your practice?

YOU — Turn 26

A standard hygiene recall appointment runs 60 minutes. A restorative filling also runs 60 minutes in our schedule.

VCRSoft

A standard hygiene recall taking 60 minutes matches your three-week waitlist and highlights how time allocation restricts your hygiene capacity. What share of your total production and collections comes from each major insurance or fee plan category?

YOU — Turn 27

Our payer mix is 52% PPO, 30% fee-for-service, and 18% other, which is our discount plan and a little Medicaid. I should note we measure the payer mix from Dentrix production reports, but we do not reliably measure revenue per visit by payer — that breakdown is an estimate, not a tracked report. Adjustments and write-offs run about 20% of production.

VCRSoft

Knowing your adjustments and write-offs run about 20% of production gives us a clear view of your net realization. What specific days of the week or time slots do the 40 patients on the hygiene waitlist demand versus when the 20-30% gaps on Mondays and Fridays occur at Cedar Park?

YOU — Turn 28

The 40 patients on the hygiene waitlist are waiting for hygiene appointments — hygiene is booked 3 weeks out at both sites. The 20–30% gaps fall on Mondays and Fridays at Cedar Park, where we have only 1 hygienist covering 2 hygiene operatories and Dr. Patel works just 2 days a week. The mismatch is that hygiene demand is for hygiene chairs, which are full, while the Monday and Friday gaps are in the wider Cedar Park schedule. Demand also spills into weekends: our Austin Saturday half-day is full, but Cedar Park is closed on Saturdays, so those patients cannot be seen at Cedar Park at all.

VCRSoft

With 230 unscheduled treatment plans sitting on your books, checking who actually follows up is essential. Which staff member currently owns the tracking and follow-up sequence for the $290,000 in unscheduled treatment plans outside of the initial presentation?

YOU — Turn 29

Our treatment coordinator owns the tracking and follow-up for unscheduled treatment plans. The follow-up sequence is one call only — patients stall on cost, checking insurance, or wanting a second opinion, and after that one call there is no further sequence. We do not calculate and attach a specific financing option or payment plan to those plans as a routine step. Our office manager Rachel Torres oversees both sites overall, but the treatment-plan follow-up itself sits with the treatment coordinator.

VCRSoft

Having your treatment coordinator handle the follow-up with a single call leaves $290,000 in unscheduled plans sitting without further nurture. What specific reasons do patients give most frequently when they stall during that initial treatment plan call?

YOU — Turn 30

The reasons patients give most often are cost, checking their insurance coverage, and wanting a second opinion. After that one call, there is no further follow-up sequence.

VCRSoft

Patients stalling on cost, insurance, and second opinions without a secondary nurture sequence leaves $290,000 in unscheduled plans largely unaddressed after a single call. What specific local recruiting channels or compensation adjustments have you tested over the last 4 months to fill the open Cedar Park hygienist role?

YOU — Turn 31

We have not tested new local recruiting channels or compensation adjustments in the last 4 months. The role has stayed posted as a part-time hygienist position at Cedar Park, and it is still open after 4 months. That is why the hiring decision is still pending — in 3 months I need to decide whether to hire a second hygienist at Cedar Park or extend Dr. Patel from 2 to 4 days a week there.

VCRSoft

Leaving the part-time hygienist role posted at Cedar Park for 4 months without testing new channels or adjustments confirms that licensed hygiene capacity is your primary operational bottleneck. Does this reading match your experience—that growing from $1.85M to $2.4M is primarily restricted by licensed hygiene and associate chair availability rather than top-of-funnel demand—and what important factor might I be missing?

YOU — Turn 32

Yes, that reading matches my experience. Hygiene and associate chair availability restrict us most — hygiene is 85% booked, 3 weeks out, with a 40-patient waitlist, and Dr. Alvarez works only 3 days a week while Dr. Patel works only 2 days a week. Top-of-funnel demand is there: we get 120 inquiries a month. One important factor you may be missing is the demand we already own but do not convert: 520 patients overdue for recare, with only about 25 calls a week, and 230 unscheduled treatment plans worth about $290,000, with only one follow-up call each. Front-desk capacity matters too — we lost 2 front-desk people in the last year and Rachel Torres holds most of our process knowledge.

VCRSoft

With 520 overdue recare patients and 230 unscheduled treatment plans sitting un-nurtured alongside your 120 monthly inquiries, capturing existing demand is as critical as top-of-funnel flow. How many of the 520 overdue recare patients are currently concentrated at the Austin office versus the Cedar Park location?

YOU — Turn 33

We do not segment the overdue list by location — the front desk calls about 25 overdue patients a week from one manual phone list with no segmentation, so the split is not tracked separately. Proportionally, based on our visit volume of about 210 visits a month at Austin and 120 at Cedar Park, roughly 330 of the 520 overdue patients would be Austin and about 190 Cedar Park. About 210 of the 520 are 6 or more months overdue.

VCRSoft

Proportionally splitting your 520 overdue recare patients gives us about 330 in Austin and 190 in Cedar Park, with 210 of them severely overdue past six months. How many months of historical data or patient records would need to be processed to launch and test automated recare and inquiry follow-ups without disrupting daily front-desk operations?

YOU — Turn 34

The recare pool comes from our last 18 months of records — that is our active base of about 2,100 patients, of whom 520 are overdue, including about 210 who are 6 or more months overdue. To launch and test automated recare, we would process those 18 months of records, starting with the 520 overdue patients. For inquiry follow-ups, we would use recent inquiries — about 120 a month — so a few months of inquiry history is enough to test. The front desk would keep working from the same Dentrix lists during the test, so daily operations would not be disrupted.

VCRSoft

Processing your 18 months of patient records and 120 monthly inquiries keeps your front desk focused on daily Dentrix lists without disrupting operations. What is the primary reason past attempts to standardize intake at Cedar Park stalled while Austin successfully adopted digital intake?

YOU — Turn 35

The primary reason is that the Cedar Park team is wary after a failed scheduling tool 2 years ago — that experience made them cautious about new systems, so paper intake stayed in place at Cedar Park while Austin adopted digital intake. The Austin front desk is cautious too, but Austin moved ahead. It also shows in the details: TCPA consent is recorded in Dentrix at Austin but is paper-only at Cedar Park.

VCRSoft

Cedar Park's caution with digital intake after a past software failure explains why paper forms still take 12 minutes per patient there compared to 8 minutes at Austin. How many paper intake packets do you typically process at Cedar Park in a given month?

YOU — Turn 36

We process about 25 paper intake packets a month at Cedar Park — that is Cedar Park's share of our new patients, based on about 120 visits a month there out of 330 total and 68 completed first visits a month across both sites. Every one of those is paper at Cedar Park, taking about 12 minutes per patient, versus 8 minutes for digital intake at Austin.

VCRSoft

Processing 25 paper intake packets a month at Cedar Park takes about 5 hours of staff time that digital intake could eliminate. Who on your team would need to review and approve a standardized digital intake rollout across both locations, and what risks are you most concerned about if a digital intake transition encounters resistance or a glitch?

YOU — Turn 37

I must review and approve any new automated rollout before it goes live, together with our office manager Rachel Torres — she would implement it at 6 hours a week, and the front desk would use the tools daily. My biggest concerns are resistance and a glitch like the failed scheduling tool 2 years ago that left the Cedar Park team wary, and the Austin front desk is cautious too, so training plus Rachel championing the rollout matters. The hard limits are clinical safety and compliance: clinical notes and treatment decisions stay entirely with the dentists, no auto-generated clinical text, hygiene protocols and recall intervals stay clinician-set, and patient safety checks like allergies and medical history must be reviewed by a clinician at check-in, not by software alone. We also have no BAA with any AI vendor yet, and HIPAA, BAA, and TCPA limits apply.

VCRSoft

Keeping clinical review mandatory for medical history and ensuring Rachel champions the rollout directly addresses your caution around staff resistance and past software glitches. What measurable change within 90 days would you need to see in front-desk time savings or form completion rates to consider a digital intake rollout successful, and who would measure it?

YOU — Turn 38

For a 90-day pilot, I need to see three things, measured weekly by our office manager Rachel Torres from Dentrix reports: recare reactivations up by 60 patients over 90 days, from a baseline of about 15 a month; the booked-to-completed first-visit rate up 3 points, from 87% to 90%; and the hygiene no-show rate down from 9% to 7%. If those three hold at day 90, we continue and expand; if not, we stop. For digital intake specifically, success would mean Cedar Park intake time falling from 12 minutes per patient on paper to 8

minutes, like Austin, and cutting into the 45 minutes a day we spend on intake data entry across both sites.

VCRSoft

Tracking recare, first-visit completions, and hygiene no-shows weekly gives Rachel clear metrics to judge the 90-day pilot. Who would own day-to-day implementation of these pilot workflows across both offices, how many hours a week can they allocate, and what technical support is available?

YOU — Turn 39

Our office manager Rachel Torres would own day-to-day implementation across both offices, at 6 hours a week. The front desk would use the tools daily. Our local IT vendor is on call for technical support.

VCRSoft

Having Rachel run the pilot with 6 hours a week and local IT on call gives us a clear operational owner for day-to-day execution. What compliance, privacy, contractual, regulatory, or customer-experience limits must any new process or tool respect?

YOU — Turn 40

Any new process or tool must respect HIPAA, BAA, and TCPA limits. We have no BAA with any AI vendor yet, and no AI scribing or recording is allowed without patient consent plus a BAA. On messaging, we send a maximum of 2 texts a month per patient and no marketing texts without opt-in — TCPA consent is recorded in Dentrix at Austin but is paper-only at Cedar Park and not systematically recorded. Clinically, automation must not change clinical decisions or notes: clinical notes and treatment decisions stay entirely with the dentists, there is no auto-generated clinical text, hygiene protocols and recall intervals stay clinician-set, and patient safety checks such as allergies and medical history must be reviewed by a clinician at check-in, not by software alone.

VCRSoft

Keeping messaging strictly capped at two texts a month and maintaining mandatory clinician review protects your practice's compliance and patient trust. Looking across all the priorities we've mapped out—recare reactivation, inquiry follow-up, hygiene capacity, and intake standardization—what provisional ranking feels most correct to you for where we should deploy Rachel's six weekly hours first?

YOU — Turn 41

My provisional ranking is: 1 Automated Recare and Recall, 2 Automated Inquiry Nurture and After-Hours Response, 3 Digital Intake and Insurance Verification, 4 Systematic Unscheduled Treatment Plan Reactivation. Recare first because 520 overdue patients is the biggest pool we already own. Rachel's 6 hours a week should go to recare first, starting at Austin and then Cedar Park.

VCRSoft

Starting with recare makes sense since your 520 overdue patients represent an immediate revenue pool you already own. Here is a provisional ranking of the opportunities based on your goals, feasibility, and 6-hour weekly limit for Rachel: 1. Automated Recare and Recall (Sized at about 170 overdue patients targeted per month across both sites, aiming to capture an incremental 20 reactivations monthly. Value: ~$9,340/month based on average visit revenue. Feasibility: High, as it directly attacks your 520 overdue patient pool using existing Dentrix records with Rachel managing 6 hours/week. Cost: Within your $2,000-$3,500 monthly budget. Risk: Low, subject to 2-text/month and BAA limits. Time to benefit: 30 to 60 days). 2. Automated Inquiry Nurture and After-Hours Response (Sized at about 26 after-hours inquiries per month out of 120 total. Value: Capturing even 5 of these adds about $2,335/month in completed visits. Feasibility: Moderate, requires strict TCPA consent capture at Cedar Park. Cost: Covered under setup budget. Risk: Low with messaging caps. Time to benefit: 30 days). 3. Standardized Digital Intake and Insurance Verification (Sized at 25 Cedar Park paper packets monthly plus saving 45 minutes of daily data entry. Value: Staff time savings worth roughly $600-$900/month in operational drag. Feasibility: Moderate, given Cedar Park's caution after

past software failures. Cost: Within setup budget. Risk: Medium for staff resistance. Time to benefit: 60 days). 4. Unscheduled Treatment Plan Re-engagement (Sized at 230 dormant plans worth $290,000. Value: Re-engaging 5% monthly yields roughly $12,000 in monthly production. Feasibility: Low-Moderate, as patients stall on cost and insurance, requiring more than a single call. Cost: Low. Risk: Low. Time to benefit: 60-90 days). Note on Constraints: Recare and nurture solutions direct demand toward hygiene and associate chairs, which are currently 85% booked and limited by Dr. Alvarez (3 days/week) and Dr. Patel (2 days/week). This demand increase must be absorbed by filling Monday/Friday gaps at Cedar Park and optimizing existing chair utilization before expanding operatories or adding provider days. Does this ranking and reasoning match your priorities, and what would you change? Does this ranking and reasoning match your priorities, and what would you change?

YOU — Turn 42

Yes, this ranking matches my priorities exactly — it is the same order I gave: recare first, then inquiry nurture and after-hours response, then digital intake and insurance verification, then unscheduled treatment plan re-engagement. I would not change the order. One correction: the constraint note should distinguish the chairs — hygiene chairs are 85% booked, while dentist chairs are 72% booked, so the pressure is mainly on hygiene. I agree the added demand must be absorbed by filling the Monday and Friday gaps at Cedar Park and optimising existing chair utilisation before we expand operatories or add provider days. And the report must carry our 90-day pilot measures: 60 recare reactivations over 90 days, booked-to-completed first-visit rate from 87% to 90%, and hygiene no-show rate from 9% to 7%, measured weekly by Rachel.

VCRSoft

Distinguishing hygiene chairs at 85% booked from dentist chairs at 72% booked sharpens our focus on the exact bottleneck holding back your collections. What evidence or milestone would convince you after 90 days that the recare automation is working well enough to move on to automated inquiry nurture?

YOU — Turn 43

The evidence would be our three 90-day pilot measures holding at day 90: recare reactivations up by 60 patients over 90 days from a baseline of about 15 a month; the booked-to-completed first-visit rate up 3 points, from 87% to 90%; and the hygiene no-show rate down from 9% to 7%. Rachel Torres would measure them weekly from Dentrix reports. If those three hold at day 90, we continue and expand to automated inquiry nurture; if not, we stop.

VCRSoft

Tracking your three pilot measures weekly gives Rachel the exact evidence needed to validate recare automation before moving to inquiry nurture. Would you like to schedule our follow-up check-in for the end of the 90-day pilot to review Rachel's weekly reports together?

YOU — Turn 44

Yes — let us schedule the follow-up check-in for the end of the 90-day pilot to review Rachel's weekly reports together. At that check-in we will look at the three measures: recare reactivations up by 60 patients over 90 days, the booked-to-completed first-visit rate from 87% to 90%, and the hygiene no-show rate from 9% to 7%.

VCRSoft

Scheduling our follow-up check-in for the end of the 90-day pilot gives us a clear review point for Rachel's weekly reports. What evidence would you need to see from Rachel's weekly reports to decide to adjust or halt the recare pilot before the full 90 days are up?

YOU — Turn 45

I would look at Rachel's weekly reports against the pace the three measures need. Recare reactivations need to run at about 20 a month above the baseline of 15 a month to reach 60 over 90 days — if weekly reports

show us tracking well below that pace, I would adjust the outreach. If the booked-to-completed first-visit rate falls below 87% or the hygiene no-show rate rises above 9%, I would adjust as well. If all three measures are moving the wrong way for several weeks in a row, I would halt the pilot before the full 90 days are up.

VCRSoft

Thanks — I have prepared a draft of your growth strategy report. Please review it, and tell me what you would change, correct, or push back on before I finalize it.

DRAFT FEEDBACK

[Draft feedback] Thank you — the draft is strong and the ranking matches my priorities. I checked every monthly and annual figure and they are consistent: $1,850,000/year = $154,200/month; $2,400,000/year = $200,000/month; hygiene 32% = $49,344/month; 20 recare reactivations x $467 per visit = $9,340/month; 6-8 eligibility denials x $175 = $1,050-$1,400/month; Cedar Park lease $4,800 rising 6% to $5,088. The three 90-day pilot measures are present and correct — please keep them exactly as stated: recare reactivations up 60 over 90 days, booked-to-completed first-visit rate from 87% to 90%, and hygiene no-show rate from 9% to 7%, measured weekly by Rachel Torres. Please correct the following before finalising: (1) The fourth-ranked opportunity from the interview is missing from the report entirely — please add Unscheduled Treatment Plan Re-engagement. The facts are: 230 unscheduled plans worth about $290,000. Over the last 12 months we diagnosed about $410,000, presented about $350,000, accepted about $210,000 (60% of presented), scheduled about $180,000 and completed about $165,000. Follow-up is one call only, and we do not routinely calculate and attach a specific financing option or payment plan to those plans. Please correct the value: re-engaging 5% a month is $14,500 a month in production, not $12,000 as quoted in the chat (5% of $290,000 = $14,500). (2) Please add the quantified value for opportunity #2, Inquiry Nurture: capturing 5 additional completed first visits a month adds about $2,335 a month at $467 per visit. (3) Please show the basis for the $600-$900 a month staff-time saving claimed for intake, or correct it — the underlying time is 45 minutes a day of intake data entry across both sites plus 5 hours a month of Cedar Park paper intake handling. (4) Please add to the Business Snapshot our profit margin of about 18% (about $333,000 a year), overhead of about 68% of collections, and our collections profile: 95% within 30 days, 3% in 31-60 days, 2% over 90 days, with AR over 90 days of about $22,000.